

India's Trusted Health Test @Home Service



NABL
ACCREDITED



Booking ID : 19852473167

Sample Collection Date : 22/Aug/2026

Aparna Divakar

Female, 33 Yrs

Healthians
Relationship
Status

Preferred
Bronze
Since
2023

A Comprehensive Health Analysis Report

AI Based Personalized Report for You



INDIA'S FIRST & ONLY CREDIBILITY CHECK FOR YOUR LAB REPORT

Check the authenticity of your lab report with machine data

Scan the QR using any QR code scanner

MACHINE & QC DETAILS

LABORATORY EQUIPMENT DASHBOARD

Aparna Divakar

Booking ID : 19852473167



Beckman AU700

Beckman Coulter : Beckman AU700

Department - Biochemistry

No. of test(s) performed - 46

Performed Test(s): RA Test Rheumatoid Arthritis Factor, Quantitative, Amylase Enzymatic, Serum, CRP (C Reactive Protein) Quantitative, Serum, HsCRP High Sensitivity CRP, Lipase, Serum, Magnesium, Serum, Potassium, Serum, Lipid Profile Advance, Liver Function Test, Iron Studies, Kidney Function Test Advance, Apolipoproteins Profile (A1, B, B/A1 Ratio)

Description: The Beckman Coulter state-of-the-art clinical chemistry analyzer manufactured in USA, delivers exceptional performance with a throughput of up to 800 tests per hour using spectrophotometry and 1,200 tests per hour with ISE, ensuring outstanding diagnostic accuracy and precision. Utilizing advanced spectrophotometric technology, it provides reliable and highly accurate results, supporting over 21 parameters that meet Six Sigma performance standards for superior laboratory efficiency and quality.

Frequency of Quality Control (QC): Twice a day (Industry best and as per NABL guidelines)



Beckman DXI800

Beckman Coulter : Beckman DXI800

Department - Immunology

No. of test(s) performed - 14

Performed Test(s): Prolactin, Testosterone Total, Insulin - Fasting, CA-125 (Ovarian Cancer Marker Test), CA-15.3 (Breast Cancer Marker Test), CEA-Carcino Embryonic Antigen (Colorectal Cancer Marker Test), Ferritin, FSH-Follicle Stimulating Hormone, LH-Luteinizing Hormone, Vitamin B12 Cyanocobalamin, Vitamin D Total-25 Hydroxy, Thyroid Profile-Total (T3, T4 & TSH Ultra-sensitive)

Description: The Beckman Coulter UniCel Dxl Immunoassay System manufactured in USA, utilizes chemiluminescent technology to deliver high precision and sensitivity, with a throughput of 400 tests per hour for efficient and reliable immunoassay testing.

Frequency of Quality Control (QC): Twice a day (Industry best and as per NABL guidelines)



Beckman DXH800

Beckman Coulter : Beckman DXH800

Department - Hematology

No. of test(s) performed - 26

Performed Test(s): Complete Blood Count Advance

Description: The Beckman Coulter hematology analyzer manufactured in USA, features automated intelligent morphology screen with advanced algorithms for accurate cell population analysis. It delivers highly reliable RBC, PLT, and WBC differentials through near native-state cellular characterization. Utilizing VCS 360, Data Fusion, and enhanced Coulter Principle technology, it ensures precise, reportable WBC differential results with exceptional diagnostic reliability.

Frequency of Quality Control (QC): Twice a day (Industry best and as per NABL guidelines)



Transasia Cube 80

Transasia : Transasia Cube 80

Department - Hematology

No. of test(s) performed - 1

Performed Test(s): ESR Automated

Description: The Cube 80 manufactured in Italy, is a fully automated bench-top analyzer designed for the direct determination of ESR. With its automation, flexibility, and reliability, the Cube 80 provides accurate and efficient ESR analysis suited for high-throughput laboratory environments.

Frequency of Quality Control (QC): Twice a day (Industry best and as per NABL guidelines)



Beckman AU700

Beckman Coulter : Beckman AU700

Department - Biochemistry

No. of test(s) performed - 1

Performed Test(s): Blood Glucose Fasting

Description: The Beckman Coulter state-of-the-art clinical chemistry analyzer manufactured in USA, delivers exceptional performance with a throughput of up to 800 tests per hour using spectrophotometry and 1,200 tests per hour with ISE, ensuring outstanding diagnostic accuracy and precision. Utilizing advanced spectrophotometric technology, it provides reliable and highly accurate results, supporting over 21 parameters that meet Six Sigma performance standards for superior laboratory efficiency and quality.

Frequency of Quality Control (QC): Twice a day (Industry best and as per NABL guidelines)

HEALTH ANALYSIS

Personalized Summary & Vital Parameters

Aparna Divakar

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

Aparna Divakar,

Congratulations, We have successfully completed your health diagnosis. This is a big step towards staying on top of your health and identify potential to improve!

10 Vital Health Parameters of a Human Body Ecosystem

Below are the health parameters which require routine checkups for primary healthcare. The view also includes *personalised information* depending on the tests you have taken.

Your Health Score

83

Out of 100

*Calculated from test reports



Thyroid Function

Thyroid Stimulating Hormone (TSH) - Ultrasensitive : 2.15 μ IU/mL

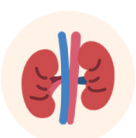
● Everything looks good



Cholesterol Total

201.4 mg/dl

● Concern



Kidney Function

Serum Creatinine : 0.78 mg/dl

● Everything looks good



Vitamin D

23.76 ng/ml

● Concern



HbA1c

4.7 %

● Everything looks good



Vitamin B12

218 pg/ml

● Concern



Liver Function

Alanine Aminotransferase (ALT/SGPT) : 107.4 U/L

● Concern



Calcium Total

9.4 mg/dl

● Everything looks good



Iron studies

Serum Iron : 95 μ g/dl

● Everything looks good



Complete Hemogram

Haemoglobin (HB) : 14.1 g/dL

● Everything looks good





New Features

Report Summary

Aparna Divakar

Booking ID 19852473167 | Sample Collection Date: 22/Aug/2026

Understanding laboratory reports can be complex, often leading to unwarranted anxiety.

At Healthians, we understand that you shouldn't have to rely on a Google search to decipher your own health report. That's why we offer comprehensive summaries that are easy to understand.

Summary of Your Health Test Results

Hello Aparna Divakar, thank you for sharing your recent health test results. It is completely natural to feel a bit concerned when seeing several markers outside of their typical ranges, but please know that these results are simply tools to help us understand your body's current needs. Your report indicates some areas of inflammation and metabolic activity that require attention, particularly regarding your liver enzymes and lipid levels. These markers often fluctuate based on lifestyle, diet, and stress, and they provide a clear roadmap for making positive adjustments.

While some of your results, such as those related to your vitamin levels and inflammatory markers, suggest that your body is working harder than it needs to, these are often manageable through consistent lifestyle changes. The elevation in certain liver and metabolic indicators suggests that your system would benefit from a more focused approach to nutrition and recovery. By addressing these imbalances now, you are taking a proactive step toward restoring your internal harmony and improving your overall well-being.

Suggestions for Your Health

Here are some suggestions to help you address the areas identified in your recent health tests:

- Liver and Metabolic Support:** Focus on a diet rich in whole, unprocessed foods, emphasizing leafy greens, cruciferous vegetables, and lean proteins. Reducing intake of refined sugars, processed carbohydrates, and alcohol can significantly help lower liver enzyme levels and improve your lipid profile.
- Nutritional Supplementation and Absorption:** Your results indicate a need to boost your Vitamin D and B12 levels. Incorporate more B12-rich foods like eggs, dairy, or fortified cereals, and consider spending time in safe, early-morning sunlight. Discussing appropriate supplementation with a healthcare provider can help ensure these levels return to a healthy range.
- Inflammation Management:** To address the markers of inflammation, prioritize consistent, moderate physical activity and stress-reduction techniques such as yoga, meditation, or deep breathing exercises. Staying well-hydrated and ensuring adequate, restorative sleep are also essential for calming your body's inflammatory response.
- Hydration and Uric Acid Balance:** To support your kidney function and help manage uric acid levels, ensure you are drinking plenty of water throughout the day. Limiting high-purine foods, such as certain red meats and sugary beverages, can also assist in maintaining a healthy balance within your system.

HEALTH ANALYSIS

HISTORICAL CHARTS

Aparna Divakar

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

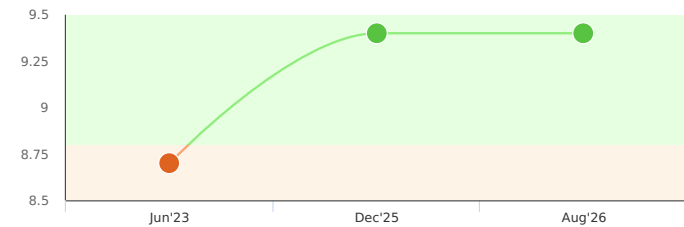
Calcium Total, Serum

Everything looks good

Your Latest result

9.4 mg/dl

22nd Aug 2026



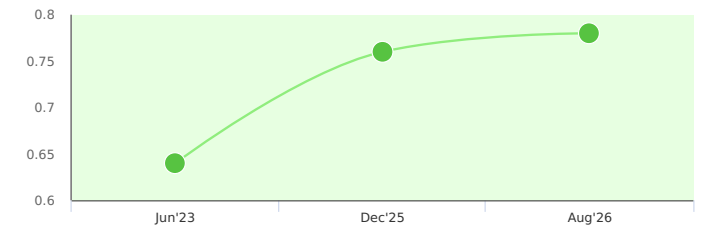
Creatinine, Serum

Everything looks good

Your Latest result

0.78 mg/dl

22nd Aug 2026



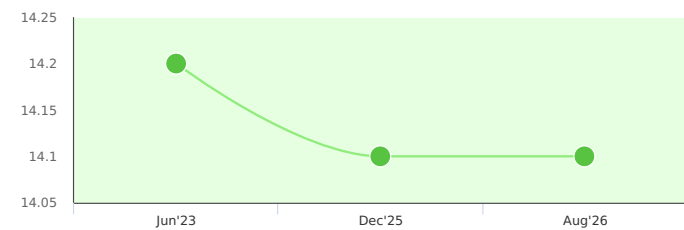
Hemoglobin Hb

Everything looks good

Your Latest result

14.1 g/dL

22nd Aug 2026



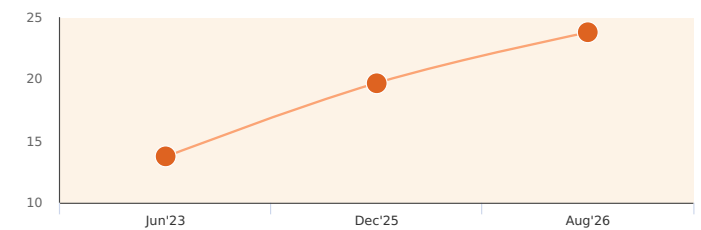
Vitamin D Total-25 Hydroxy

Concern

Your Latest result

23.76 ng/ml

22nd Aug 2026



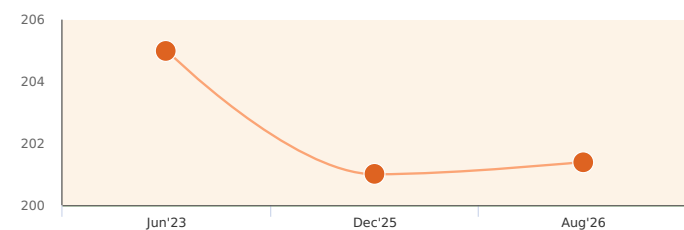
Cholesterol-Total, Serum

Concern

Your Latest result

201.4 mg/dl

22nd Aug 2026



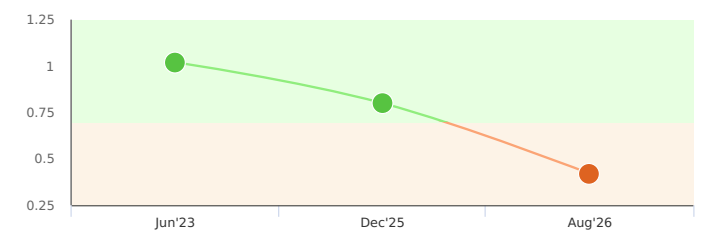
SGOT/SGPT Ratio

Concern

Your Latest result

0.42 Ratio

22nd Aug 2026



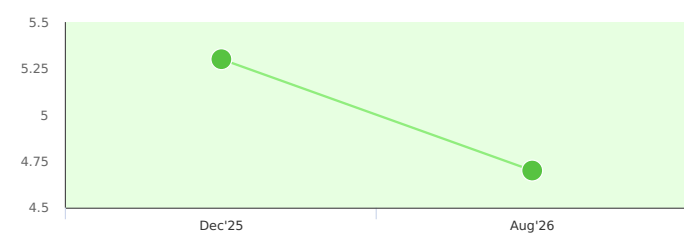
Glycated Hemoglobin (HbA1c)

Everything looks good

Your Latest result

4.7 %

22nd Aug 2026



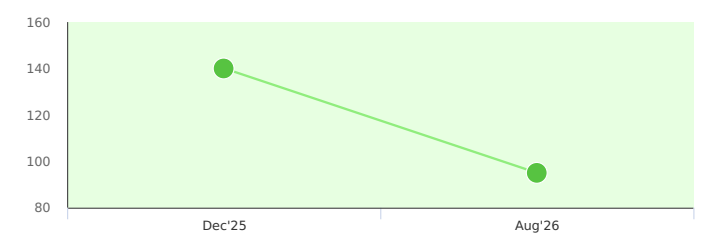
Iron, Serum

Everything looks good

Your Latest result

95 ug/dl

22nd Aug 2026



HEALTH ANALYSIS
COMPARATIVE CHARTS

Aparna Divakar
Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

↑ Increase From Last Time ↓ Decrease From Last Time

Total Parameters :35
(As per latest result)

23 Everything Looks Good

1 Borderline

11 Concern

		2 Years Ago		8 Months Ago	Latest Result 22 Aug 2026
Test Name	Risk Area	Booking ID 8349770569 24 Jun 2023 10:30 AM	Booking ID 16124726558 10 Dec 2025 09:00 AM		
Cholesterol–Total, Serum 0 –200 Normal Range		205 mg/dl	↓ 201 mg/dl	↑ 201.4 mg/dl High (Concern)	
HDL Cholesterol Direct 40 –60 Normal Range	--	43.9 mg/dl	↓ 42.6 mg/dl	↑ 52.7 mg/dl Everything Looks Good	
LDL Cholesterol –Direct 0 –100 Normal Range		139 mg/dl	-	↓ 133.1 mg/dl High (Concern)	
Triglycerides, Serum 0 –150 Normal Range	--	89 mg/dl	↑ 232.5 mg/dl	↓ 129.5 mg/dl Everything Looks Good	
VLDL 0 –30 Normal Range	--	-	-	16 mg/dl Everything Looks Good	
Alkaline Phosphatase, Serum 33 –98 Normal Range		98.5 u/L	↓ 95.4 u/L	↑ 104.6 u/L High (Concern)	
GGTP (Gamma GT) 5 –38 Normal Range		105.6 u/L	↑ 123.3 u/L	↑ 221.3 u/L High (Concern)	
Proteins, Serum 6.6 –8.3 Normal Range	--	7 gm/dl	↑ 7.48 gm/dl	↑ 7.52 gm/dl Everything Looks Good	
SGOT/AST 3 –35 Normal Range		63.4 u/L	↓ 22.8 u/L	↑ 45.6 u/L High (Concern)	
SGPT/ALT 3 –35 Normal Range		62.1 u/L	↓ 28.5 u/L	↑ 107.4 u/L High (Concern)	
ESR Automated 0 –12 Normal Range		39 mm/1st hour	-	↓ 19 mm/1st hour High (Concern)	
Hemoglobin Hb 12.0 –15.0 Normal Range	--	14.2 g/dL	↓ 14.1 g/dL	14.1 g/dL Everything Looks Good	

HEALTH ANALYSIS

COMPARATIVE CHARTS

Aparna Divakar

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

↑ Increase From Last Time

↓ Decrease From Last Time

Total Parameters :35
(As per latest result)

23 Everything Looks Good

1 Borderline

11 Concern

		2 Years Ago		8 Months Ago	Latest Result 22 Aug 2026
Test Name	Risk Area	Booking ID 8349770569 24 Jun 2023 10:30 AM	Booking ID 16124726558 10 Dec 2025 09:00 AM		
Platelet Count Thrombocyte count 150 -410 Normal Range	--	329 10^3/μl	↑ 330 10^3/μl	↓ 300 10^3/μl Everything Looks Good	
T3, Total Tri Iodothyronine 0.87 -1.78 Normal Range	--	-	1.45 ng/ml	↓ 1.27 ng/ml Everything Looks Good	
T4, Total Thyroxine 5.48 -14.28 Normal Range	--	-	7.24 ug/dl	↑ 9.11 ug/dl Everything Looks Good	
TSH Ultra - Sensitive 0.38 -5.33 Normal Range	--	-	1.87 μIU/mL	↑ 2.15 μIU/mL Everything Looks Good	
Iron, Serum 60 -180 Normal Range	--	-	140 ug/dl	↓ 95 ug/dl Everything Looks Good	
Calcium Total, Serum 8.8 -10.6 Normal Range	--	8.7 mg/dl	↑ 9.4 mg/dl	9.4 mg/dl Everything Looks Good	
Chlorides, Serum 101 -109 Normal Range	--	105 mmol/L	↓ 102 mmol/L	↑ 106 mmol/L Everything Looks Good	
Creatinine, Serum 0.3 -1.0 Normal Range	--	0.64 mg/dl	↑ 0.76 mg/dl	↑ 0.78 mg/dl Everything Looks Good	
Phosphorus-Inorganic, Serum 2.5 -4.5 Normal Range	--	2.8 mg/dl	-	↑ 3.7 mg/dl Everything Looks Good	
Sodium, Serum 136 -146 Normal Range	--	138 mmol/L	138 mmol/L	↑ 140 mmol/L Everything Looks Good	
Urea, Serum 17 -43 Normal Range		13 mg/dl	13 mg/dl	↑ 14.8 mg/dl Low (Concern)	
Uric Acid, Serum 2.6 -6.0 Normal Range		6.4 mg/dl	↓ 5.9 mg/dl	↑ 6.9 mg/dl High (Concern)	

HEALTH ANALYSIS
COMPARATIVE CHARTS

Aparna Divakar
Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

↑ Increase From Last Time ↓ Decrease From Last Time

Total Parameters :35
(As per latest result)

23 Everything Looks Good

1 Borderline

11 Concern

		2 Years Ago		8 Months Ago
Test Name	Risk Area	Booking ID 8349770569 24 Jun 2023 10:30 AM	Booking ID 16124726558 10 Dec 2025 09:00 AM	Latest Result 22 Aug 2026
URINE KETONE Negative -Negative Normal Range	--	-	Negative	Negative
Glycated Hemoglobin (HbA1c) 4.2 -5.7 Normal Range	--	-	5.3 %	↓ 4.7 % Everything Looks Good
Apolipoproteins A1, Serum 105 -205 Normal Range	--	-	-	125.6 mg/dl Everything Looks Good
Apolipoproteins B, Serum 55 -130 Normal Range	--	-	-	95.6 mg/dl Everything Looks Good
Potassium, Serum 3.5 -5.5 Normal Range	--	3.95 mmol/L	-	↑ 4.53 mmol/L Everything Looks Good
Ferritin 11 -306.8 Normal Range	--	-	72.4 ng/ml	↓ 57 ng/ml Everything Looks Good
Vitamin D Total-25 Hydroxy 30 -100 Normal Range		13.71 ng/ml	↑ 19.68 ng/ml	↑ 23.76 ng/ml Low (Concern)
CRP (C Reactive Protein) Quantitative, Serum 0 -5 Normal Range	--	-	-	3.76 mg/L Everything Looks Good
HsCRP High Sensitivity CRP 0 -1.0 Normal Range		-	-	3.11 mg/L High (Concern)
Amylase Enzymatic, Serum 22 -80 Normal Range	--	-	-	64.6 u/L Everything Looks Good
Lipase, Serum 0 -67 Normal Range	--	-	-	31 u/L Everything Looks Good

Patient Name	: Aparna Divakar	Barcode	: D0342207
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:30PM
Referred By	: Self	Report Generated On	: 22/Aug/2026 08:15PM
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓
Sample Type	: Whole Blood EDTA	Report Status	: Final Report

DEPARTMENT OF BIOCHEMISTRY HBA1C

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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HbA1c - Glycosylated Hemoglobin

HbA1c (Glycosylated Hemoglobin)	4.70	%	4.2 - 5.7
Method: HPLC			
Average Estimated Glucose	88.19		
Method: Calculated			

INTERPRETATION:

AS PER AMERICAN DIABETES ASSOCIATION (ADA):

REFERENCE GROUP

Non diabetic
At Risk (Prediabetes)
Diagnosing Diabetes

GLYCOSYLATED HEMOGLOBIN (HBA1c) in %

<5.7
5.7 – 6.4
>= 6.5
Age > 19 Years
Goals of Therapy: < 7.0
Actions Suggested: >8.0
Age < 19 Years
Goal of therapy: <7.5

Therapeutic goals for glycemic control

REMARKS

- HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
 - HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
 - Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapson, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
 - HbA1c may be increased in patients with polycythemia or post-splenectomy.
 - Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.
 - Trends in HbA1c are a better indicator of diabetic control than a solitary test. 7. Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.
 - HbA1c target in pregnancy is to attain level <6 % .
 - HbA1c target in paediatric age group is to attain level < 7.5 %.
- Method : Ion-exchange high-performance liquid chromatography (HPLC).
Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2023

DR RAMESH VISHWANATH
MBBS, MD PATHOLOGY
HEALTHIANS LABS



SIN No: D0342207

Patient Name	: Aparna Divakar	Barcode	: D0342207	
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:01PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 03:00PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Sodium Fluoride	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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Fasting Blood Sugar

Glucose, Fasting	84.4	mg/dl	70 - 100
Method: Hexokinase			
Machine: BECKMAN DXC 700AU			

American Diabetes Association Reference Range :

Normal	: < 100 mg/dl
Impaired fasting glucose(Prediabetes)	: 100 - 126 mg/dl
Diabetes	: >= 126 mg/dl

Conditions that can result in an elevated blood glucose level include: Acromegaly, Acute stress (response to trauma, heart attack, and stroke for instance), Chronic kidney disease, Cushing syndrome, Excessive consumption of food, Hyperthyroidism, Pancreatitis

A low level of glucose may indicate hypoglycemia, a condition characterized by a drop in blood glucose to a level where first it causes nervous system symptoms (sweating, palpitations, hunger, trembling, and anxiety), then begins to affect the brain (causing confusion, hallucinations, blurred vision, and sometimes even coma and death). A low blood glucose level (hypoglycemia) may be seen with: Adrenal insufficiency, Drinking excessive alcohol, Severe liver disease, Hypopituitarism, Hypothyroidism, Severe infections, Severe heart failure, Chronic kidney (renal) failure, Insulin overdose, Tumors that produce insulin (insulinomas), Starvation.

Dr. Ramesh Vishwanath

DR RAMESH VISHWANATH
MBBS, MD PATHOLOGY
HEALTHIANS LABS



Patient Name	: Aparna Divakar	Barcode	: D0342207
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:00PM
Referred By	: Self	Report Generated On	: 22/Aug/2026 09:13PM
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓
Sample Type	: Serum	Report Status	: Final Report

DEPARTMENT OF BIOCHEMISTRY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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Amylase - Serum

AMYLASE	64.6	U/L	22 - 80
Method: CNPG3			
Machine: BECKMAN DXC 700AU			

Amylase is produced by exocrine pancreas and also by the salivary glands. it is used to evaluate pancreatic function and also used in the diagnosis and management of pancreatitis.

Diseases resulting in elevation of plasma alpha-amylase include: acute pancreatitis, parotitis, alcoholism, renal insufficiency and diseases such as viral hepatitis, AIDS, abdominal typhoid, sarcoidosis and trauma to the upper abdomen. There is also a detectable increase in amylase after an ERCP procedure. In acute pancreatitis, amylase increases 5-6 hours after the onset of symptoms and remains elevated for 2-5 days. The increase in plasma activity does not reflect disease severity and conversely, extensive destruction of the pancreas may not cause a significant increase in the plasma concentration of pancreatic alpha-amylase.

Lipase - Serum

LIPASE	31	U/L	<67
Method: Colorimetric			
Machine: BECKMAN DXC 700AU			

Lipase is produced in the acinar cells of the pancreas and is responsible for the hydrolysis of water-insoluble long chain fatty acid esters of glycerol. Lipase measurement in serum and plasma is used exclusively for the investigation of pancreatic disorders, usually pancreatitis. Serum lipase may be elevated in acute pancreatitis, acute episodes of chronic pancreatitis and obstructive pancreatitis

Slight elevations are also frequently present in diabetic ketoacidosis, viral hepatitis, epidemic parotiditis, abdominal typhoid and sarcoidosis, due to involvement of the pancreas.

Apolipoproteins profile (A1,B,B/A1 ratio)

APO-A1	125.6	mg/dl	105 - 205
Method: Immunoturbidimetry			
Machine: BECKMAN DXC 700AU			
APO-B	95.6	mg/dl	55 - 130
Method: Immunoturbidimetry			
Machine: BECKMAN DXC 700AU			
Apo B /Apo A-1 Ratio	0.76		

Apolipoprotein A (apo-A; also known as Apo A-1) is the major protein (90%) of HDL. Apolipoprotein B (apo B) is major protein component of low-density lipoprotein (LDL) and is important in regulating cholesterol synthesis and metabolism.

Use:

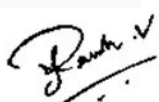
To evaluate the risk of CAD: Levels of Apo A-1 are inversely associated with premature cardiovascular disease and peripheral vascular disease. The ratio of Apo B to Apo A has greater sensitivity and specificity for CAD than individual lipid or lipoproteins. To evaluate atherosclerotic disease and to detect Tangier disease.

Apo B/A-1 ratio:

Low risk: Male: 0.4 -0.99, Female: 0.3 -0.89

Average risk: Male: 1.0-1.59, Female: 0.9-1.49

Twice average risk: Male: >1.6, Female: >1.5


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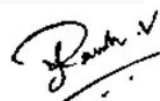
Patient Name	: Aparna Divakar	Barcode	: D0342207	
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM	
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Referred By	: Self	Report Generated On	: 22/Aug/2026 09:13PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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Ref. Wallach's Interpretation of Diagnostic Tests Pathways to Arriving at a Clinical Diagnosis TENTH EDITION



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Patient Name	: Aparna Divakar	Barcode	: D0342207	
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:00PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 08:35PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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C-Reactive Protein (CRP) -Quantitative

C-REACTIVE PROTEIN (CRP) (QUANTITATIVE)	3.76	mg/L	<5
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Method: Immunoturbidimetric
Machine: BECKMAN DXC 700AU

C-reactive protein (CRP) is one of the most sensitive acute-phase reactants for inflammation. Measuring changes in the concentration of CRP provides useful diagnostic information about the level of acuity and severity of a disease. Unlike ESR, CRP levels are not influenced by hematologic conditions such as anemia, polycythemia etc.

Increased levels are consistent with an acute inflammatory process. After onset of an acute phase response, the serum CRP concentration rises rapidly (within 6-12 hours and peaks at 24-48 hours) and extensively. Concentrations above 100 mg/L are associated with severe stimuli such as major trauma and severe infection (sepsis).

hs CRP (C-Reactive Protein high sensitive)

HS-CRP (HIGH SENSITIVITY C-REACTIVE PROTEIN)	3.11	mg/L	< 1.0
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Method: Latex Particle Immunoturbidimetry
Machine: BECKMAN DXC 700AU

High sensitivity C-reactive protein, when used in conjunction with other clinical laboratory evaluation of acute coronary syndromes, may be useful as an independent marker of prognosis for recurrent events in patients with stable coronary disease or acute coronary syndrome. Hs-CRP levels should not be substituted for assessment of traditional cardiovascular risk factors. Patients with persistently unexplained, marked elevation of hs-CRP after repeated testing should be evaluated for non-cardiovascular etiologies.

Clinical significance :

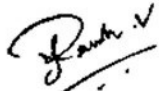
Hs-CRP measurements may be used as an independent risk marker for the identification of individuals at risk for future cardiovascular disease. Elevated CRP values may be indicative of prognosis of individuals with acute coronary syndromes, and may be useful in the management of such individuals.

Iron study

Serum Iron	95.0	ug/dl	60 - 180
Method: TPTZ Machine: BECKMAN DXC 700AU			
UIBC	278.50	ug/dl	155 - 355
Method: Nitroso-PSAP Machine: BECKMAN DXC 700AU			
Serum Total Iron Binding Capacity (TIBC)	373.5	µg/dl	250 - 400
Method: Calculated			
Transferrin Saturation %	25.44	%	15 - 50
Method: Calculated			

Iron participates in a variety of vital processes in the body varying from cellular oxidative mechanisms to the transport and delivery of oxygen to body cells. It is a constituent of the oxygen-carrying chromoproteins, haemoglobin and myoglobin, as well as various enzymes, such as cytochrome oxidase and peroxidases.

Serum iron may be increased in hemolytic, megaloblastic and aplastic anemias, and in hemochromatosis acute leukemia, lead poisoning, pyridoxine deficiency, thalassemia, excessive iron therapy, and after repeated transfusions. Drugs causing increased serum iron include chloramphenicol, cisplatin,


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Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

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Test Name	Value	Unit	Bio. Ref Interval
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estrogens (including oral contraceptives), ethanol, iron dextran, and methotrexate. Iron can be decreased in iron-deficiency anemia, acute and chronic infections, carcinoma, nephrotic syndrome hypothyroidism, in protein- calorie malnutrition and after surgery. Diurnal variation is seen in serum iron levels with normal values obtained in the midmorning, low values in midafternoon and very low values near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Transferrin saturation represents the number of iron-binding sites that are occupied. It is a better index of iron stores than serum iron alone. Transferrin saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).

Rheumatoid Factor (RA) - Quantitative - Serum

RHEUMATOID FACTOR	<10	IU/mL	<14
Method: Immunoturbidimetry			
Machine: BECKMAN DXC 700AU			

The rheumatoid factor (RF) test is primarily used to help diagnose rheumatoid arthritis (RA) and to help distinguish RA from other forms of arthritis or other conditions that cause similar symptoms. A cyclic citrullinated peptide (CCP) antibody test can help diagnose RA in someone who has joint inflammation with symptoms that suggest but do not yet meet the criteria of RA and may be ordered along with RF or if the RF result is negative. The RF test must be interpreted in conjunction with a person's symptoms and clinical history. A negative RF test does not rule out RA.

About 20% of people with RA will have very low levels of or no detectable RF. In these cases, a CCP antibody test may be positive and used to confirm RA. Positive RF test results may also be seen in 1-5% of healthy people and in some people with conditions such as: Sjogren syndrome, scleroderma, systemic lupus erythematosus (lupus), sarcoidosis, endocarditis, tuberculosis, syphilis, HIV/AIDS, hepatitis, infectious mononucleosis, cancers such as leukemia and multiple myeloma, parasitic infection, or disease of the liver, lung or kidney.

Potassium - Serum

Serum Potassium	4.53	mmol/L	3.5 - 5.5
Method: ISE (Indirect)			
Machine: BECKMAN DXC 700AU			

Potassium is a primary intracellular ion, only 2 % is extracellular, high intracellular concentration is maintained by a Na- K ATPase pump, which continuously transports potassium into the cell against a concentration gradient. The pump is a critical factor in maintaining and adjusting the ionic gradients, on which nerve impulse transmission and contractility of cardiac and skeletal muscle depends. In acidemia, potassium moves out of the cells, in alkalemia, potassium moves into the cells. Hypokalemia inhibits aldosterone production and hyperkalemia stimulates aldosterone production.


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Test Name	Value	Unit	Bio. Ref Interval
Magnesium -Serum			
MAGNESIUM	1.9	mg/dl	1.9 - 2.5

Method: Xylidyl blue, tris buffer
Machine: BECKMAN DXC 700AU

Test Interpretation

Magnesium is primarily an intracellular ion associated with GI absorption and renal excretion. It helps in the diagnosis and monitoring of hypomagnesemia and hypermagnesemia especially in renal failure or GI disorders.

Increased levels

seen in patients taking diuretics, antacids, laxatives, parenteral nutrition, magnesium for eclampsia or premature labor, renal failure, dehydration with diabetic coma before treatment, Hypothyroidism.

Decreased levels

seen in malabsorption, abnormal loss of GI fluids, chronic pyelonephritis, renal tubular acidosis, antibiotics, phosphate depletion, extra cellular fluid volume expansion. Nutritional causes -starvation, Alcoholism. Endocrine causes -hyperthyroidism, hypoparathyroidism. Metabolic causes- excessive lactation, third trimester of pregnancy. Other causes- severe burns, lytic tumors of bone, toxemia of pregnancy, sepsis.



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Test Name	Value	Unit	Bio. Ref Interval
Lipid Profile Advance			
Total Cholesterol Method: Enzymatic Machine: BECKMAN DXC 700AU	201.4	mg/dl	Desirable : <200 Borderline: 200-239 High : >=240
Serum Triglycerides Method: Enzymatic Machine: BECKMAN DXC 700AU	129.5	mg/dl	Desirable : <150 Borderline high : 150-199 High : 200-499 Very high : >= 500
Serum HDL Cholesterol Method: Enzymatic immuno inhibition Machine: BECKMAN DXC 700AU	52.7	mg/dl	40 - 60
Serum LDL Cholesterol Method: Enzymatic Machine: BECKMAN DXC 700AU	133.1	mg/dl	Optimal : <100 near /above Optimal:100 - 129 Borderline High:130 - 159 High : 160 - 189 Very High :>=190
Serum VLDL Cholesterol Method: Calculated Machine: BECKMAN DXC 700AU	16.0	mg/dl	<30
Total CHOL / HDL Cholesterol Ratio Method: Calculated	3.82	Ratio	3.30 - 4.40
LDL / HDL Cholesterol Ratio Method: Calculated	2.53	Ratio	Desirable/Low Risk: 0.5-3.0 Line/Moderate Risk: 3.0-6.0 Elevated/High Risk: >6.0
HDL / LDL Cholesterol Ratio	0.40	Ratio	Desirable/Low Risk : 0.5 - 3.0 Border Line/Moderate Risk : 3.0 - 6.0 Elevated/High Risk: > 6.0
Non-HDL Cholesterol Method: Calculated	148.7	mg/dl	0.0 - 160.0


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Test Name	Value	Unit	Bio. Ref Interval
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* You have received this report as a part of your 'Dyslipidemia' evaluation.

Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency.

Dyslipidemias means increase in the level of one or more of the following:

a. Total Cholesterol

b. The "bad" cholesterol or low density lipoprotein (LDL) and/or triglyceride concentrations

Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood

* Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation.

* Healthians labs report biological reference intervals (normal ranges) in accordance to the recommendations of The National Cholesterol Education Program (NCEP) & Adult Treatment Panel IV (ATP IV) Guidelines providing the most desirable targets of various circulating lipid fractions in the blood. NCEP recommends that all adults above 20 years of age must be screened for abnormal Lipid levels

*NCEP recommends the assessment of 3 different samples drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays. Hence a single result of Lipid Profile may not be adequate for clinical decision making. Healthians' counselling team will reach you shortly to explain implications of your report. You may reach out to customer support helpline as well.

*NCEP recommends lowering of LDL Cholesterol as the primary therapeutic target with lipid lowering agents, however, if triglycerides remain >200 mg/dL after LDL goal is reached, set secondary goal for non-HDL cholesterol (total minus HDL) 30 mg/dL higher than LDL goal.

*High Triglyceride and low HDL levels are independent risk factors for Coronary Heart disease and requires further clinical consultation.

*Healthians lab performs direct LDL measurement which is more appropriate and may vary from other lab reports which provide calculated LDL values.

Liver Function Test (LFT)

Serum Bilirubin, (Total)	0.92	mg/dl	0.3 - 1.2
Method: Diazo			
Machine: BECKMAN DXC 700AU			
Serum Bilirubin, (Direct)	0.18	mg/dl	0 - 0.2
Method: Diazo			
Machine: BECKMAN DXC 700AU			
Serum Bilirubin, (Indirect)	0.75	mg/dl	0.0 - 0.8
Method: Calculated			
Aspartate Aminotransferase (AST/SGOT)	45.60	U/L	3 - 35
Method: IFCC			
Machine: BECKMAN DXC 700AU			
Alanine Aminotransferase (ALT/SGPT)	107.4	U/L	3 - 35
Method: IFCC			
Machine: BECKMAN DXC 700AU			
Alkaline Phosphatase (ALP)	104.60	U/L	33-98


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Test Name	Value	Unit	Bio. Ref Interval
Method: IFCC AMP Buffer Machine: BECKMAN DXC 700AU			
Gamma Glutamyl Transferase (GGT)	221.3	U/L	5- 38
Method: IFCC Machine: BECKMAN DXC 700AU			
Serum Total Protein	7.52	gm/dl	6.6 - 8.3
Method: Biuret Machine: BECKMAN DXC 700AU			
Serum Albumin	4.33	g/dl	3.5 - 5.2
Method: Bromo Cresol Green(BCG) Machine: BECKMAN DXC 700AU			
Serum Globulin	3.19	gm/dl	3.0 - 4.2
Method: Calculated			
Albumin/Globulin Ratio	1.36	Ratio	1.2 - 2.0
Method: Calculated			
SGOT/SGPT Ratio	0.42	Ratio	0.7 - 1.4
Method: Calculated			

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels results from increased bilirubin production (eg hemolysis and ineffective erythropoiesis); decreased bilirubin excretion (eg; obstruction and hepatitis); and abnormal bilirubin metabolism (eg; hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in viral hepatitis; drug reactions, alcoholic liver disease conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion reaction & a common metabolic condition termed Gilbert syndrome.

AST levels increase in viral hepatitis, blockage of the bile duct ,cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. Ast levels may also increase after a heart attack or strenuous activity. ALT is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health. Elevated ALP levels are seen in Biliary Obstruction, Osteoblastic Bone Tumors, Osteomalacia, Hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, paget's disease, Rickets, Sarcoidosis etc.

Serum total protein, also known as total protein, is a biochemical test for measuring the total amount of protein in serum..Protein in the plasma is made up of albumin and globulin. Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma, Waldenstrom's disease. Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic - Human serum albumin is the most abundant protein in human blood plasma. It is produced in the liver. Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting etc.

Please note that there is a change in the Biological Reference Interval for Alkaline Phosphatase (ALP) parameter. (Dated: 12.02.2026).

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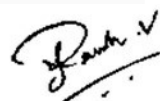
Test Name	Value	Unit	Bio. Ref Interval
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Kidney Function Test Advance

Serum Creatinine Method: Jaffes Kinetic Machine: BECKMAN DXC 700AU	0.78	mg/dl	0.3 - 1.0
GFR, ESTIMATED Method: Calculated (CKD-EPI)	102.79	mL/min/1.73m2	
Serum Uric Acid Method: Uricase Machine: BECKMAN DXC 700AU	6.9	mg/dl	2.6-6.0
Serum Calcium Method: Arsenazo Machine: BECKMAN DXC 700AU	9.4	mg/dl	8.8 - 10.6
Serum Phosphorus Method: Phosphomolybdate complex Machine: BECKMAN DXC 700AU	3.7	mg/dl	2.5 - 4.5
Serum Sodium Method: ISE (Indirect) Machine: BECKMAN DXC 700AU	140	mmol/L	136 - 146
Serum Chloride Method: ISE (Indirect) Machine: BECKMAN DXC 700AU	106	mmol/L	101 - 109
Blood Urea Method: Urease Machine: BECKMAN DXC 700AU	14.8	mg/dl	17 - 43
Blood Urea Nitrogen (BUN) Method: Calculated	6.9	mg/dl	7.92 - 20.03
Bun/Creatinine Ratio Method: Calculated	8.87	Ratio	
Urea/Creatinine Ratio Method: Calculated	18.97	Ratio	

The kidneys play a vital role in the excretion of waste products and toxins such as urea, creatinine and uric acid, regulation of extracellular fluid volume, serum osmolality and electrolyte concentrations, as well as the production of hormones like erythropoietin and 1,25 dihydroxy vitamin D and renin. Assessment of renal function is important in the management of patients with kidney disease or pathologies affecting renal function. Tests of renal function have utility in identifying the presence of renal disease, monitoring the response of kidneys to treatment, and determining the progression of renal disease.

Urea is synthesized in the liver as the final product of protein and amino acid metabolism. Urea synthesis is therefore dependent on daily protein intake and endogenous protein metabolism.



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Creatinine is a metabolic product of creatine and phosphocreatine, which are both found almost exclusively in muscle.

Uric Acid is the major product of purine catabolism in humans. Uric acid levels are used to monitor the treatment of gout.

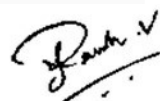
Measurement of calcium is used in the diagnosis and treatment of parathyroid disease, a variety of bone diseases, chronic renal disease, urolithiasis and tetany. Phosphorus levels are increased in acute or chronic renal failure with decreased GFR.

Sodium is an electrolyte, and it helps regulate the amount of water in and around the cells & the balance of chemicals in the body called acids and bases.

Chloride is a negatively charged ion that works with other electrolytes such as potassium, sodium, and bicarbonate, to help regulate the amount of fluid in the body and maintain the acid-base balance.

EGFR-National kidney Disease Education program recommends the use of MDRD equation to predict/ estimate GFR in adults (>=20 years) with Chronic kidney disease. Calculated by serum creatinine may be less accurate due to certain factor like race muscle mass, diet and certain drugs in such case EGFR should be calculated using serum cystatin C.

Age (Years)	Male ML/min/1.73m ²	Female ML/min/1.73m ²
17-24	93-131	—
25-34	78-146	—
35-44	74-138	65-123
45-54	74-129	58-110
55-64	69-122	50-110
65-74	61-114	46-105
75-84	52-102	48-85



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Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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NAFLD score

NAFLD score	-4.25
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Test Description :

- NAFLD Fibrosis Score (NFS) is a validated non-invasive tool used to assess the likelihood of advanced liver fibrosis in patients with suspected non-alcoholic fatty liver disease (NAFLD).
- It is derived from a combination of clinical and laboratory parameters.

NAFLD Score	Correlated Fibrosis Severity	Interpretation
< -1.455	F0-F2	low likelihood of advanced fibrosis
-1.455 – 0.675	Indeterminate score	further evaluation recommended
> 0.675	F3-F4	high likelihood of advanced fibrosis

- Non-alcoholic fatty liver disease (NAFLD) has become the most common cause of chronic liver disease worldwide and has a strong association with insulin resistance and metabolic syndrome.
- NAFLD encompasses a wide spectrum of liver pathology, ranging from simple steatosis without fibrosis to non-alcoholic steatohepatitis with varying stages of fibrosis and cirrhosis.
- In the absence of decompensated cirrhosis, liver biopsy remains the only reliable means to determine prognosis based on the severity of fibrosis. However, liver biopsy is an expensive and invasive procedure associated with a number of complications and prone to sampling error.
- Several non-invasive diagnostic scoring tests such as NAFLD fibrosis score (NFS), Enhanced liver fibrosis score (ELF) etc., have been developed to help determining the severity of liver fibrosis in lieu of liver biopsy.
- NAFLD fibrosis score (NFS) has a high negative predictive value and is useful as a first line screening tool to rule out advanced fibrosis, thereby reducing the need for liver biopsy in many patients. However, results should always be interpreted in conjunction with clinical findings and where indicated, additional investigations (e.g., imaging or elastography).



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Test Name	Value	Unit	Bio. Ref Interval
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CA 15.3 (Cancer Antigen 15.3)- Breast Cancer

CA 15-3	9.50	U/mL	0.0–23.5
Method: Chemiluminescent			
Machine: UniCel DxI 800			

Clinical Significance: CA 15-3 is a glycoprotein expressed on various adenocarcinoma, especially breast. It is a high molecular weight polymorphic epithelial mucin. CA 15-3 is the most useful as an aid in management of Breast Carcinoma. It is useful in monitoring therapy and progression in metastatic breast cancer patients.

1. Increased levels can be seen in:

- Malignant (including but not limited to)- metastatic breast cancer, Pancreas, Lung, Ovarian, Colorectal, and Liver cancers with less specificity.
- Non-cancerous (Benign) conditions (including but not limited to)- Liver Disease (Cirrhosis, Hepatitis), Benign Breast Diseases, Endometriosis, Pelvic Inflammatory Disease, and Sarcoidosis

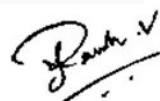
2. Decreased levels can be seen in:

- Effective therapy or post removal of the tumor

Note:

- The diagnostic efficiency of any tumor marker depends on a variety of factors such as its sensitivity, specificity, positive predictive value (PPV) and negative predictive value (NPV). A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- Serial testing should be done to identify elevated levels caused due to transient physiological/ pathological factors
- Samples from patients receiving mouse monoclonal antibodies for diagnosis or therapy, may show falsely elevated or depressed values under specific conditions. Heterophilic antibodies in human serum, certain medications & non-specific protein binding may interfere with the test results
- Tumor marker test results are influenced by assay methods, reagent sensitivity and specificity, and should not be used interchangeably. If in the course of monitoring a patient, the assay method used is changed, additional testing should be carried out to confirm baseline values. All the results need to be followed up for correlation on similar platforms and methodologies

CA-125 (Ovarian Cancer Marker Test)



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Test Name	Value	Unit	Bio. Ref Interval
CA-125 (Ovarian Cancer Marker Test)	11.4	U/mL	< 35
Method: CLIA			
Machine: UniCel DxI 800			

Clinical Significance: CA-125 may be used in association with transvaginal ultrasound, for early detection of ovarian cancer in women. It is also recommended as an adjunct in distinguishing benign from malignant suspicious pelvic masses, particularly in postmenopausal women. Measurements may also be used to monitor response to chemotherapeutic response.

1. Increased levels can be seen in:

- Malignant (including but not limited to)- Fallopian tube tumors, Non-Mucinous Epithelial Ovarian Carcinoma, Cervical Adenocarcinoma, Endometrial Adenocarcinoma, and Squamous Cell Carcinomas of Vulva or Cervix, Trophoblastic Tumors, Non-Hodgkin Lymphoma involving Pleuro-Pericardial or Peritoneum, Cancers of Pancreas, Liver, and Lung.
- Non-cancerous (Benign) conditions (including but not limited to)- conditions that affect the Endometrium, Pregnancy, Menstruation, Endometriosis, Uterine Fibroids, Pleural Effusion or Inflammation like- Congestive Heart Failure, Peritoneal Effusion, Pelvic Inflammatory Disease, Cirrhosis, Severe Liver Necrosis, other diseases and disorders of the GI Tract, Liver Failure, and Renal Failure.

2. Decreased levels can be seen in:

- Postmenopausal women
- Effective therapy or post removal of the tumor

Note:

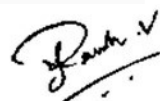
- The diagnostic efficiency of any tumor marker depends on a variety of factors such as its sensitivity, specificity, positive predictive value (PPV) and negative predictive value (NPV). A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- Serial testing should be done to identify elevated levels caused due to transient physiological/ pathological factors
- Samples from patients receiving mouse monoclonal antibodies for diagnosis or therapy, may show falsely elevated or depressed values under specific conditions. Heterophilic antibodies in human serum, certain medications & non-specific protein binding may interfere with the test results
- Tumor marker test results are influenced by assay methods, reagent sensitivity and specificity, and should not be used interchangeably. If in the course of monitoring a patient, the assay method used is changed, additional testing should be carried out to confirm baseline values. All the results need to be followed up for correlation on similar platforms and methodologies

CEA (Carcino Embryonic Antigen)

CARCINO EMBRYONIC ANTIGEN (CEA)	1.81	ng/mL	Non smokers < 3.00 Smokers < 5.00
Method: CLIA			
Machine: UniCel DxI 800			

Note:

1. This test is not recommended for cancer screening in the general population.
2. False negative / positive results are observed in patients receiving mouse monoclonal antibodies for diagnosis or therapy.



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Patient Name	: Aparna Divakar	Barcode	: D0342207	
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:00PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 04:53PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
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3. Patients with confirmed carcinoma may show normal pre-treatment CEA levels. Hence this assay, regardless of level, should not be interpreted as absolute evidence for the presence or absence of malignant disease. The assay value should be used in conjunction with findings from clinical evaluation and other diagnostic procedures.

4. Persistently elevated CEA levels are usually indicative of progressive malignant disease and poor therapeutic response

5. The results obtained with different analytical techniques and different equipments cannot be used interchangeably due to difference in assay methods and reagent specificity.

6. In course of monitoring, the assay method preferably should not be changed.

Comment:

CEA is produced by cells during embryonic and fetal life and production ceases at birth. A very low serum concentration can be detected in healthy individuals. Increased CEA levels can be found in certain cases of cancer (Colorectal, breast, lung cancer etc.), but also in nonmalignant diseases. Serum CEA levels decrease after treatment & increase in the event of cancer recurrence, residual disease and metastases. The CEA test can be used as an additional test for prognosis and therapeutic monitoring of patients with diagnosed malignant carcinomas. Smokers have higher values than non smokers.

Clinical Use

Monitoring patients with Colorectal, Gastrointestinal, Lung & Breast carcinoma

Diagnosis of occult metastatic disease and / or residual disease

Increased values

* Colorectal, gastrointestinal lung, pancreas, breast, and ovarian cancer

* Some diseases like cirrhosis, pancreatitis, kidney failure, inflammatory bowel disease, peptic ulcer disease, chronic obstructive pulmonary disease (COPD), or an obstructed bile duct.

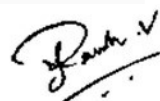
* **Heavy smoking affects the test results.**

* Please note test values may vary depending on the assay method used.

Clinical Significance: CEA is a glycoprotein involved in cell adhesion. It is normally produced in fetal gastrointestinal tissue, and its production stops before birth. Healthy adults normally have very low levels of CEA in their blood. Elevated levels may be associated with various malignant and non-malignant conditions.

1. Increased levels can be seen in:

- Malignant (including but not limited to)- Colorectal Carcinoma, Breast Cancer, Pancreatic Cancer, Lung Cancer, Medullary Thyroid Carcinoma, Ovarian/Cervical Carcinoma.
- Non-cancerous (Benign) conditions (including but not limited to)- Active Nonmalignant inflammatory diseases (especially of GI Tract) (Ulcerative Colitis, Regional Enteritis, Cholecystitis, Diverticulitis, Peptic Ulcers, Chronic Pancreatitis, Inflammatory Bowel Disease), Liver Disease (Alcoholic, Cirrhosis, Chronic Active Hepatitis, Obstructive Jaundice), Renal Disorder, Fibrocystic Disease of Breast, Smokers, Breast Cysts, Chronic Pulmonary Diseases, Mucinous Cystadenoma of the Ovary/Appendix, Lungs Issues, Ulcers, etc.



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Patient Name	: Aparna Divakar	Barcode	: D0342207	
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Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

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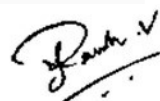
Test Name	Value	Unit	Bio. Ref Interval
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2. Decreased levels can be seen in:

- Effective therapy or post removal of the tumor

Note:

- The diagnostic efficiency of any tumor marker depends on a variety of factors such as its sensitivity, specificity, positive predictive value (PPV) and negative predictive value (NPV). A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- Serial testing should be done to identify elevated levels caused due to transient physiological/ pathological factors
- Samples from patients receiving mouse monoclonal antibodies for diagnosis or therapy, may show falsely elevated or depressed values under specific conditions. Heterophilic antibodies in human serum, certain medications & non-specific protein binding may interfere with the test results
- Tumor marker test results are influenced by assay methods, reagent sensitivity and specificity, and should not be used interchangeably. If in the course of monitoring a patient, the assay method used is changed, additional testing should be carried out to confirm baseline values. All the results need to be followed up for correlation on similar platforms and methodologies



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Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:00PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 05:03PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
Ferritin			
Ferritin	57.0	ng/ml	11-306.8
Method: CLIA			
Machine: UniCel DxI 800			
Ferritin estimation is useful in the diagnosis of iron deficiency anemia and iron overload. Elevated ferritin levels also are observed in acute and chronic liver disease, chronic renal failure and in some types of neoplastic disease. Increased levels seen in hemochromatosis, frequent blood transfusions with packed RBCs and alcoholic liver disease. Decreased levels seen in heavy menstrual bleeding, poor absorption of iron, iron deficiency anaemia and long term GI bleed. Ferritin is an acute phase reactant and thus may be increased with inflammation, chronic infection, liver disease, auto-immune disorders and some type of cancers. Ferritin is not used to detect or monitor these conditions.			
Follicle Stimulating Hormone, FSH			
Follicle Stimulating Hormone (FSH)	3.46	mIU/mL	Mid Follicular Phase: 3.85-8.78
Method: CLIA			Mid Cycle Peak: 4.54-22.51
Machine: UniCel DxI 800			Mid Luteal Phase: 1.79-5.12
			Post Menopausal: 16.74-113.59
FSH stimulates follicular growth and in conjunction with LH, stimulates estrogen secretion and ovulation. Human FSH is secreted by the gonadotropic cells of the anterior lobe of the pituitary gland in response to gonadotropin releasing hormone (GnRH) from the medial basal hypothalamus. Abnormal FSH levels are interpreted with increased or decreased levels of other fertility hormones such as LH, estrogens, progesterone, and testosterone. Increased FSH levels are associated with menopause and primary ovarian hypofunction in females and primary hypogonadism in males. Decreased FSH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males. Normal or decreased FSH levels are associated with polycystic ovary disease in females.			


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Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:00PM
Referred By	: Self	Report Generated On	: 22/Aug/2026 08:42PM
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓
Sample Type	: Serum	Report Status	: Final Report

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Test Name	Value	Unit	Bio. Ref Interval
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Homocysteine

HOMOCYSTEINE	10.0	μmol/L	5 - 13
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Method: Immunoturbidimetric
Machine: BECKMAN DXC 700AU

Clinical Significance

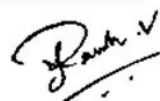
Homocysteine is linked to increased risk of premature coronary artery disease, stroke and Thromboembolism and additionally alzheimer's disease, Osteoporosis, Venous Thrombosis, schizophrenia, Cognitive Deficiency and pregnancy complications.

High Values

Elevated Homocysteine may be due to increasing age, genetic traits, drugs, Renal dysfunction, and dietary deficiency of vitamins or smoking. To lower your Homocysteine, eat more green vegetables, stop smoking, Alcohol. Folic Acid helps lowering elevated levels.

Caution while interpretation

To get most accurate results, it is mandatory to separate serum immediately. In separated serum, Homocysteine remains stable for at least 48 hours at room temperature.



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DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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INSULIN (FASTING)

INSULIN (FASTING)	22.70	μIU/mL	1.9 - 23
Method: CLIA			
Machine: UniCel DxI 800			

Insulin: insulin levels are tested:

To help evaluate insulin production by the beta cells in the pancreas; to help diagnose the presence of an insulin-producing tumor in the islet cells of the pancreas (insulinoma); to help determine the cause of low blood glucose (hypoglycemia); to help identify insulin resistance, or to help determine when a type 2 diabetic might need to start taking insulin to supplement oral medications



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Age/Gender : 33Y 0M 0D /Female
Order Id : 19852473167
Referred By : Self
Customer Since : 24/Jun/2023
Sample Type : Serum

Barcode : D0342207
Sample Collected On : 22/Aug/2026 10:29AM
Sample Received On : 22/Aug/2026 03:00PM
Report Generated On : 22/Aug/2026 05:05PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF IMMUNOLOGY

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Test Name	Value	Unit	Bio. Ref Interval
Luteinizing Hormone, LH			
Luteinizing Hormone (LH)	4.970	mIU/ml	Mid Follicular Phase: 2.12-10.89
Method: CLIA			Mid Cycle Peak: 19.18-103.03
Machine: UniCel DxI 800			Mid Luteal Phase: 1.20-12.86
			Post Menopausal: 10.87-58.64

Human Luteinizing Hormone (LH) is a glycoprotein hormone secreted by the anterior pituitary gland. LH, FSH and other steroid hormones play important role in regulating the ovulation and ovarian functions during the menstrual cycle. Maturation of an ovarian follicle and its oocyte begins during the end of the preceding menstrual cycle. When FSH is released by the pituitary gland, the ovarian follicle undergoes rapid increases. This rapid increase is generally believed to trigger the rapid rise and peak of LH activity at mid-cycle, the LH surge. Ovulation begins twelve to twenty-four hours after the LH surge. The wall of the enlarged follicle ruptures and the mature ovum is extruded. Within two days the LH concentration returns to its baseline level. A concomitant rise in progesterone level initiates the luteal phase, which lasts approximately fourteen days. During this time, if the mature ovum is not fertilized, a new follicle enters the maturation route marking the beginning of the next menstrual cycle. In light of the characteristic fluctuation of LH levels during the menstrual cycle, rapid and sensitive detection of LH is useful in the diagnosis and treatment of infertility and in the detection of the LH surge to predict the time of ovulation. As the onset of the LH surge precedes ovulation by approximately thirty hours, its determination has been used successfully to time oocyte retrieval for in vitro fertilization and to assist in the timing of artificial insemination. INCREASED LEVELS are seen in Luteal phase of menstrual cycle, Primary hypogonadism, Gonadotropin secreting pituitary tumours, Menopause. DECREASED LEVELS are seen in Hypothalamic gonadotropin releasing hormone deficiency, Pituitary LH deficiency, Ectopic steroid hormone production, Gonadotropin releasing hormone analog treatment.

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Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:00PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 04:47PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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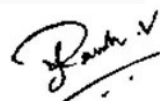
Prolactin

Prolactin	9.9	ng/ml	3.34-26.72
Method: CLIA			
Machine: UniCel DxI 800			

Prolactin values that exceed the reference values may be due to macroprolactin (prolactin bound to immunoglobulin). Macroprolactin should be evaluated if signs and symptoms of hyperprolactinemia are absent or pituitary imaging studies are not informative.

Prolactin deficiencies in normal individuals are rare. Pathologic causes of hyper-prolactinemia include: PRL secreting pituitary adenomas (prolactinomas), functional and organic diseases of the hypothalamus, hypothyroidism, renal failure, and ectopic tumors. Elevated levels of PRL may be observed in cases of primary hypothyroidism due to an increased secretion of TRH (stimulates PRL release) accompanied by decreased serum T4 levels and increased serum thyroid stimulating hormone concentrations. Hyper-prolactinemia has also been associated with the inhibition of ovarian steroidogenesis, follicle maturation, and secretion of luteinizing hormone and follicle stimulating hormone.

Note:- Prolactin levels will vary over a 24-hour period, rising during sleep and peaking in the early morning.



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Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:00PM
Referred By	: Self	Report Generated On	: 22/Aug/2026 04:35PM
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓
Sample Type	: Serum	Report Status	: Final Report

DEPARTMENT OF IMMUNOLOGY

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Test Name	Value	Unit	Bio. Ref Interval
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Thyroid Profile (Total T3, T4, TSH)

Tri-Iodothyronine (T3, Total)	1.27	ng/ml	0.87 - 1.78
Method: CLIA Machine: UniCel DxI 800			
Thyroxine (T4, Total)	9.11	ug/dl	5.48-14.28
Method: CLIA Machine: UniCel DxI 800			
Thyroid Stimulating Hormone (TSH)-Ultrasensitive	2.153	μIU/mL	0.38 - 5.33
Method: CLIA Machine: UniCel DxI 800			

Pregnancy interval

Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)

First trimester	0.1 - 2.5
Second trimester	0.2 - 3.0
Third trimester	0.3 - 3.0

Healthians recommends that the following potential sources of variation should be considered while interpreting thyroid hormone results:

1. Thyroid hormones undergo rhythmic variation within the body this is called circadian variation in TSH secretion: Peak levels are seen between 2-4 AM. Minimum levels seen between 6-10 AM. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. Total T3 and T4 levels are seen to have physiological rise during pregnancy and in patients on steroid treatment.
4. T4 may be normal even in the presence of hyperthyroidism under the following conditions : T3 thyrotoxicosis, Hypoproteinemia related reduced binding, during intake of certain drugs (eg Phenytoin, Salicylates etc)
5. Neonates and infants have higher levels of T4 due to increased concentration of TBG
6. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
7. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
8. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones.
9. Various drugs influence the levels of thyroid hormones such as L-Dopa, Lithium, Glucocorticoids, Phenytoin etc.
10. Healthians recommends evaluation of unbound fractions, that is free T3 (fT3) and free T4 (fT4) for clinic-pathologic correlation, as these are the metabolically active forms.

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Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Testosterone - Total

Testosterone, Total	0.81	ng/mL	< 0.1-0.75
Method: CLIA			
Machine: UniCel DxI 800			

Test Interpretation

*Total Testosterone reference intervals as per Tanner Stage:

Tanner Stage	Reference Interval – Male (ng/mL)	Reference Interval – Female (ng/mL)
Stage 1	0.02 – 0.23	0.02 – 0.10
Stage 2	0.05 – 0.70	0.05 – 0.30
Stage 3	0.15 – 2.8	0.10 – 0.30
Stage 4	1.05 – 5.45	0.15 – 0.40
Stage 5	0.65 – 8.0	0.10 – 0.40

Tanner staging should be assessed by the treating physician/ pediatrician.

Testosterone in males is secreted by adult leydig cells and is controlled by luteinizing hormone (LH). In females, it is produced in the ovaries, adrenal gland, and peripheral fatty tissues and has a serum concentration that is approximately 10-fold less than in males. In healthy adults, ~44% of circulating testosterone is bound to sex hormone binding globulin (SHBG), 50- 55% is bound to albumin & cortisol-binding globulin, & 2-3% is free; later three being bioavailable forms.

Increased in:

Adrenal and ovarian tumors, CAH, Polycystic ovary syndrome, ovarian stromal hyperthecosis, Idiopathic hirsutism

Decreased in:

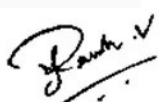
Primary hypogonadism (e.g., orchiectomy), Secondary hypogonadism (e.g., hypopituitarism), Testicular feminization, Klinefelter syndrome, Cirrhosis, Chronic renal disease, Estrogen therapy

Limitations- Testosterone exhibits significant circadian variations, thus early-morning samples are recommended. Because SHBG concentrations can be influenced by many factors, tests assessing bioavailable and free testosterone is helpful in clinical decision-making.

Ref: Tietz textbook of clinical chemistry and molecular diagnostics, 5th edition

Wallach's interpretation of diagnostic tests, 11th edition

Note: Kindly note change in reference ranges with effect from 18/08/2023.


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DEPARTMENT OF IMMUNOLOGY

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Test Name	Value	Unit	Bio. Ref Interval
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Vitamin B12

VITAMIN B12	218	pg/ml	222 - 1439
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Method: CLIA

Machine: UniCel DxI 800

Vitamin B12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency.

Patients taking vitamin B12 supplementation may have misleading results. A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

Please note change in Biological Reference interval.

Vitamin D, 25-Hydroxy

VITAMIN D (25 - OH VITAMIN D)	23.76	ng/ml	30 - 100
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Method: CLIA

Machine: UniCel DxI 800

Biological Reference Ranges:

Deficiency	Below 20 ng/ml
Insufficiency	20 - 30 ng/ml
Sufficiency	30 - 100 ng/ml
Toxicity	Above 100 ng/ml.

The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs).

The reference ranges discussed in the preceding are related to total 25-OHD; as long as the combined total is 30 ng/mL or more, the patient has sufficient vitamin D. Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20–30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL.

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Patient Name	: Aparna Divakar	Barcode	: D0342207
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:30PM
Referred By	: Self	Report Generated On	: 22/Aug/2026 08:42PM
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓
Sample Type	: Whole Blood EDTA	Report Status	: Final Report

DEPARTMENT OF HAEMATOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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Complete Blood Count Advanced

Haemoglobin (HB)	14.1	g/dL	12.0-15.0
Method: Photometric Measurement Machine: BECKMAN DXH 800			
Total Leucocyte Count (TLC)	7.3	10 ³ /uL	4.0-10.0
Method: Coulter Principle Machine: BECKMAN DXH 800			
Hematocrit (PCV)	42.6	%	36.0-46.0
Method: Calculated Machine: BECKMAN DXH 800			
Red Blood Cell Count (RBC)	4.90	10 ⁶ /μl	3.80-4.80
Method: Coulter Principle Machine: BECKMAN DXH 800			
Mean Corp Volume (MCV)	87.1	fL	83.0-101.0
Method: Derived from RBC Histogram Machine: BECKMAN DXH 800			
Mean Corp Hb (MCH)	28.8	pg	27.0-32.0
Method: Calculated Machine: BECKMAN DXH 800			
Mean Corp Hb Conc (MCHC)	33.1	g/dL	31.5-34.5
Method: Calculated Machine: BECKMAN DXH 800			
RDW - CV	13.2	%	11.6-14.0
Method: Derived from RBC Histogram Machine: BECKMAN DXH 800			
RDW - SD	40.30	fL	39.0-46.0
Method: Derived from RBC Histogram Machine: BECKMAN DXH 800			
Mentzer Index	17.78	Ratio	
Method: Calculated			
RDWI	234.64	Ratio	
Method: Calculated			
Green and king index	71	Ratio	
Method: Calculated			

Differential Leucocyte Count

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Referred By : Self
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Sample Type : Whole Blood EDTA

Barcode : D0342207
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Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF HAEMATOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
Neutrophils Method: VCS Technology Machine: BECKMAN DXH 800	43.1	%	40 - 80
Lymphocytes Method: VCS Technology Machine: BECKMAN DXH 800	44.5	%	20-40
Monocytes Method: VCS Technology Machine: BECKMAN DXH 800	9.7	%	02 - 10
Eosinophils Method: VCS Technology Machine: BECKMAN DXH 800	2.4	%	01 - 06
Basophils Method: VCS Technology Machine: BECKMAN DXH 800	0.3	%	00 - 02
Absolute Leucocyte Count			
Absolute Neutrophil Count (ANC) Method: Calculated Machine: BECKMAN DXH 800	3.15	10 ³ /uL	2.0-7.0
Absolute Lymphocyte Count (ALC) Method: Calculated Machine: BECKMAN DXH 800	3.25	10 ³ /uL	1.0-3.0
Absolute Monocyte Count Method: Calculated Machine: BECKMAN DXH 800	0.71	10 ³ /uL	0.2-1.0
Absolute Eosinophil Count (AEC) Method: Calculated Machine: BECKMAN DXH 800	0.18	10 ³ /uL	0.02-0.5
Absolute Basophil Count Method: Calculated Machine: BECKMAN DXH 800	0.02	10 ³ /uL	0.02 - 0.10
Platelet Count(PLT) Method: Coulter Principle Machine: BECKMAN DXH 800	300	10 ³ /μl	150-410
MPV Method: Derived from PLT Histogram Machine: BECKMAN DXH 800	8.2	fL	7 - 9

DR RAMESH VISHWANATH
MBBS, MD PATHOLOGY
HEALTHIANS LABS



Patient Name	: Aparna Divakar	Barcode	: D0342207	
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:30PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 08:42PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF HAEMATOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
NLR (Neutrophil Lymphocyte Ratio) Method: Calculated	0.97	Ratio	
PLR (Platelet Lymphocyte Ratio) Method: Calculated	92	Ratio	

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making. This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely causes, making it necessary to distinguish between them.

If the quotient of the mean corpuscular volume divided by the red blood cell count is then 13, thalassemia is more likely. If the result is greater than 13, then iron-deficiency anaemia is more likely.

Green and King Index used to differentiate IDA from thalassemia trait value >65 is likely to be Iron Deficiency Anemia and value <65 Beta Thalassemia Trait. For RDW Value >220 more likely to be Iron Deficiency Anemia and value <220 more likely to be Beta Thalassemia Trait.

The neutrophil to lymphocyte ratio (NLR) is considered to be a sensitive indicator of infection, inflammation and sepsis, and a robust prognostic marker of disease severity.

The platelet-to-lymphocyte ratio (PLR) is a marker revealing shifts in platelet & lymphocyte cell counts due to acute inflammatory and pro-thrombotic states.



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Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:30PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 05:03PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF HAEMATOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
ERYTHROCYTE SEDIMENTATION RATE (ESR)			
ESR	19	mm/1st hour	0-12
Method: Modified Westergren Method			
Machine: Vesmatic cube 80			

Ramesh V

DR RAMESH VISHWANATH
MBBS, MD PATHOLOGY
HEALTHIANS LABS



Patient Name	: Aparna Divakar	Barcode	: D0342207	
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:55PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 04:30PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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Urine Routine & Microscopy Extended

PHYSICAL EXAMINATION

Colour	Yellow		Pale Yellow
Method: Visual			
Volume	20.00	mL	
Method: Visual			
Appearance	Clear		Clear
Method: Visual			

CHEMICAL EXAMINATION

Specific Gravity	1.015		1.001 - 1.035
Method: Dipstick-Ion exchange			
Machine: Urine Analyzer(URI-PLUS600)			
pH	5.0		4.5 - 7.5
Method: Automated/ strip Dipstick-Double indicator			
Machine: Urine Analyzer(URI-PLUS600)			
Glucose	Negative		Negative
Method: Automated/ strip Dipstick-oxidase peroxidase			
Machine: Urine Analyzer(URI-PLUS600)			
Urine Protein	Negative		Negative
Method: Automated/ strip Dipstick-Bromophenol blue			
Machine: Urine Analyzer(URI-PLUS600)			
Ketones	Negative		Negative
Method: Automated/ strip Sodium nitroprusside			
Machine: Urine Analyzer(URI-PLUS600)			
Urobilinogen	Normal		Normal
Method: Automated/ strip Dipstick-Ehrlichs Test			
Machine: Urine Analyzer(URI-PLUS600)			
Bilirubin	Negative		Negative
Method: Automated / strip (Diazonium salt)/ Fouchets test			
Machine: Urine Analyzer(URI-PLUS600)			
Nitrite	Negative		Negative
Method: Automated/ strip Dipstick-Griess test			
Machine: Urine Analyzer(URI-PLUS600)			
Blood	Negative		Nil
Method: Automated/ strip Dipstick-Peroxidase			

Dr. Ramesh Vishwanath

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Referred By	: Self	Report Generated On	: 22/Aug/2026 04:30PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
Machine: Urine Analyzer(URI-PLUS600)			
Leucocyte Esterase	Negative		Nil
Method: Dipstick- Esterase			
Machine: Urine Analyzer(URI-PLUS600)			
MICROSCOPIC EXAMINATION			
Pus Cells	2-3	/HPF	0 - 5
Method: Microscopy			
Epithelial cells	3-4	/HPF	0 - 5
Method: Microscopy			
RBCs	Nil	/HPF	Nil
Method: Microscopy			
Casts	Nil		Nil
Method: Microscopy			
Crystals	Nil		Nil
Method: Microscopy			
Bacteria	Absent		Absent
Method: Microscopy			
Yeast Cell	Nil		
Others (Non Specific)	Nil		
Method: Microscopy			

The main indication for testing for glucose in urine is detection of unsuspected diabetes mellitus or follow-up of known diabetic patients. Renal glycosuria accounts for 5% of cases of glycosuria in general population.

Proteinuria can be seen in nephrotic syndrome, pyelonephritis, heavy metal poisoning, tuberculosis of kidney, interstitial nephritis, cystinosis, Fanconi syndrome, rejection of kidney transplant. Hemodynamic proteinuria is transient and can be seen in high fever, hypertension, heavy exercise, congestive cardiac failure, seizures, and exposure to cold. Post-renal proteinuria is caused by inflammatory or neoplastic conditions in renal pelvis, ureter, bladder, prostate, or urethra.

Ketonuria can be seen in uncontrolled Diabetes mellitus with ketoacidosis, Glycogen storage disorder, starvation, persistent vomiting in children, weight reduction program, fever in children, severe thyrotoxicosis, pregnancy and protein calorie malnutrition.

Presence of bilirubin in urine indicates conjugated hyperbilirubinemia (obstructive or hepatocellular jaundice). Bile salts along with bilirubin can be detected in urine in cases of obstructive jaundice. Normally about 0.5-4 mg of urobilinogen is excreted in urine in 24 hours. Therefore, a small amount of urobilinogen is normally detectable in urine. Increased urobilinogen in urine can be seen due to hemolysis, megaloblastic anemia and haemorrhage in tissues. Decreased urobilinogen can be seen in obstructive jaundice, reduction of intestinal bacterial flora, neonates and following antibiotic treatment. The presence of abnormal number of intact red blood cells in urine is called as hematuria. It implies presence of a bleeding lesion in the urinary tract. Hematuria can be seen in glomerular diseases like Glomerulonephritis, Berger's disease, lupus nephritis, Henoch-Schonlein purpura, non glomerular diseases like Calculus, tumor, infection, tuberculosis, pyelonephritis, hydronephrosis, polycystic kidney disease, trauma, after strenuous physical exercise, diseases of prostate (benign hyperplasia of prostate, carcinoma of prostate).

Nitrites are not present in normal urine. Ingested nitrites are converted to nitrate and excreted

in urine. If gram-negative bacteria (e.g. E.coli, Salmonella, Proteus, Klebsiella, etc.) are present in urine, they will reduce the nitrates to nitrites through the action of bacterial enzyme nitrate reductase. As E. coli is the commonest organism causing urinary tract infection, this test is helpful as a screening test for urinary tract infection.

Some organisms like Staphylococci or Pseudomonas do not reduce nitrate to nitrite and therefore in such infections nitrite test is negative.

Leucocyte esterase test detects esterase enzyme released in urine from granules of leucocytes. Thus the test is positive in pyuria.

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Referred By	: Self	Report Generated On	: 22/Aug/2026 04:30PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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Ramesh V

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Patient Name	: Aparna Divakar	Barcode	: D0342207	
Age/Gender	: 33Y 0M 0D /Female	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:29PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 02:49PM	
Customer Since	: 24/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF SEROLOGY

Healthians Life Maximiser Plus Health Checkup Package - Female Couple

Test Name	Value	Unit	Bio. Ref Interval
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Hepatitis B Virus Surface Antigen (HBsAg)

Hepatitis B Virus Surface Antigen	Non Reactive		Non- Reactive
Method: Rapid chromatographic immunoassay			

This is a screening test ,If requires confirmation or correlation can be done by HBsAg neutralisation test .HBV viral load can be done post clinician evalutaion. This is a visual, rapid immuno chromatographic,one step immunoassay based on antigen capture for the qualitative detection of Hepatitis B Surface Antigen (HBsAg) in Human Serum or Plasma. Samples containing mouse monoclonal antibodies or heterophile antibodies can give falsely reactive or non-reactive results. False Reactive results can be obtained due to the presence of other antigens or elevated levels of RF factor.

*** End Of Report ***


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HEALTHIANS LABS



Terms & Conditions

1. Test results released pertain to the sample/specimen submitted by the customer.
2. Test results are dependent on the quality of the samples received by the Lab.
3. The tests are carried out in the lab with the presumption that the sample/specimen belongs to the patient named or identified in the bill/test request form/booking ID.
4. The laboratory reports are not diagnostic. The reported results are for information only, and are subject to clinical correlation and interpretation by the consulting physician.
5. Test results may show interlaboratory variations depending upon different analyzers, methodologies, corresponding kits, biological reference intervals, and physiological status of the customer.
6. No internal data shall be shared with the customer.
7. Liability of Healthians Labs for deficiency of services or other errors and omissions shall be limited to the fee paid by the customer for the relevant laboratory services only.
8. This report is not subject to use for any medico-legal purposes.
9. The company has the right to cancel those tests which are not possible as per the received sample or clinical reference or information provided.
10. The reported values of the specific analytes are subject to the maximum reportable range as per the analyzers & kits.
11. Some tests may be performed in partner labs (outsourced). Expedient Healthcare Marketing Private Limited (Healthians Labs) makes no express or implied warranties regarding the technical precision, absolute accuracy, or Turnaround Times (TAT) of tests performed in partnered labs. Expedient Healthcare Marketing Private Limited (Healthians Labs) explicitly disclaims all liability for financial losses, operational downtime, schedule disruptions, or damages resulting from test performed at partner labs (outsourced). The name of the partner labs (outsourced) shall be mentioned explicitly on the respective reports.

ADVISORY

Health Advisory

Aparna Divakar

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

27.01 Body Mass Index

5'3" Height (ft/in)

70 Weight (kgs.)



Physical Activity

No Data



Smoke

No Data



Food Preference

No Data



Alcohol

No Data



Medication

No Data



Family History

No Data



Blood Pressure

No Data



Pulse Rate

No Data



Waist (In Cm)

0cm



Hip Circumference (In Cm)

No Data



SPO2 Levels

No Data



Sugar Levels

No Data

Additional Remarks :

NA

SUGGESTED NUTRITION

SUGGESTED NUTRITION

Do's

- Have a balanced diet that includes whole grains, pulses, dairy, fruits, vegetables, nuts and healthy fats
- Include calcium rich foods like milk, yoghurt, cheese and green, leafy vegetables
- Include Brazil nuts, sesame seeds, sunflower seeds
- Include fruits like apples, berries and melons in your diet
- Take vitamin C rich foods like citrus fruits, strawberries and green, leafy vegetables
- Include nuts like almonds, walnuts and seeds like flaxseeds, sunflower seeds
- Include fresh garlic and fenugreek seeds in your diet
- Have cruciferous vegetables like broccoli, cauliflower and cabbage

Dont's

- Limit sugar intake
- Decrease intake of colas and sugary drinks
- Reduce caffeine intake
- Avoid flavoured and seasoned foods
- Avoid saturated fats, trans fats, oily and greasy foods like cakes, creamy or fried foods
- Avoid red meat and organ meats
- Avoid refined sugars, processed foods and bakery items
- Avoid high cholesterol and calorie dense foods
- Avoid the use of oil and avoid sauces and dressings

SUGGESTED LIFESTYLE

SUGGESTED LIFESTYLE

Do's

- Have regular exposure to sunlight
- Stay active and maintain ideal weight
- Sleep well at night and do relaxing activities
- Maintain ideal weight
- Lose weight gradually and stay active

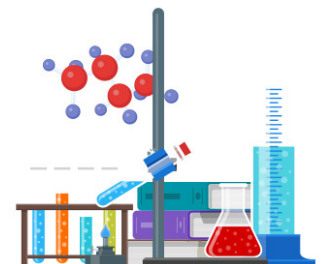
Dont's

- Avoid long periods of inactivity
- Avoid overexertion without having food or drink
- Avoid smoking and alcohol
- Avoid strenuous exercises
- Avoid overworking or being stressed for long time
- Avoid having long gaps in meals or skipping meals
- Avoid late night heavy meals
- Avoid overeating or calorie rich food

SUGGESTED FUTURE TESTS

SUGGESTED FUTURE TESTS

- Vitamin D Total-25 Hydroxy - **Every 2 Month**
- Calcium Total, Serum - **Every 2 Month**
- Complete Hemogram - **Every 2 Month**
- Peripheral Smear Examination By Pathologist - **Every 2 Month**
- Vitamin B12 Cyanocobalamin - **Every 1 Month**
- Folic Acid - **Every 1 Month**
- Electrocardiogram - **Every 1 Month**



HEALTH ADVISORY

Suggestions for Health & Well-being

Aparna Divakar

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

PHYSICAL ACTIVITY

PHYSICAL ACTIVITY

Physical activities can vary from Regular walks (Brisk or normal), Jogging , Sports, Stretching, Yoga to light weight lifting etc. It is recommended to partake in physical activity at least 30 minutes a day for 3-4 days a week.

If regular workout is difficult, then we can adapt changes such as using stairs instead of lift/escalators and doing household work!



BALANCED DIET

A balanced diet is the key to healthy lifestyle. Include Whole grains, vegetables, whole fruits, nuts, seeds, beans, plant oils in your diet.

It is recommended to always have a high protein breakfast and a light dinner. Avoid items such as processed foods, potatoes and high calorie/sugar products. Don't forget to drink water regularly!

BALANCED DIET

STRESS MANAGEMENT

STRESS MANAGEMENT

Managing stress is an essential part of well-being. Some day to day changes can help such as having sufficient sleep (6-8 hours), indulging yourself in meditation, positive attitude towards lifestyle, using humor, traveling, talking to people whom you feel comfortable with and making time for hobbies by doing what you love to do.



BMI

BMI recommended range is 18.5 to 24.9. Your BMI is **27.01**, which is on a higher side.

Please follow recommended diet and maintain a healthy lifestyle and try to keep your BMI within the desired range. Keeping the right BMI for your body helps prevent many untimely diseases and goes a long way.

BMI CHART



SUGGESTED BMI

Supplement Suggestions

Aparna Divakar

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

Your test report has indicated that you have certain deficiencies in your body which may hamper your health & wellbeing in the longer run.

In order to fulfill the gaps in nutrition and promote a healthier body we suggest you the following supplements mentioned below:

Deficiency/Out of Range Parameter(s)	Suggested Supplement
LDL Cholesterol -Direct	HEARTUP
LH-Luteinizing Hormone	FEM-UP

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



HEARTUP

Improve your heart health, the natural way!

Lower your blood pressure and give your heart a healthy beat with HEART-UP, an all-natural supplement developed especially to promote good heart health. Harnessing the remedial properties of garlic, peepal, and cinnamon, this clinically proven natural supplement lowers your blood pressure, thus ensuring a healthy heart, which in turn means a healthy you.

If left unchecked, hypertension can lead to:

- Heart Failure
- Kidney Diseases
- Heart Attack
- Stroke
- Vascular Dementia

Infused with the ages-proven goodness of all-natural ingredients, HEART-UP is the perfect supplement to help you control hypertension or high blood pressure without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in HEART-UP present the following benefits:

Arjun Tree Extract

Reduces the risk of heart diseases with anti-hypertensive properties

Garlic

Helps manage blood pressure and lowers cholesterol

Peepal

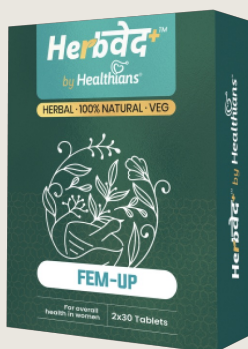
Purifies the blood and boosts cardiac health

Jatamansi

Helps in alleviating anxiety, thus reducing the risk of high blood pressure

Cinnamon

Has anti-viral properties, reduces blood pressure, and lowers the risk of Type 2 diabetes



FEM-UP

For Pink of Health in Women

Polycystic ovarian disease (PCOD) giving you sleepless nights? Let the power of Ayurveda become your shield against PCOD effects with FEM-UP. An all-natural supplement that helps in alleviating the debilitating impact of PCOD.

Infused with the natural goodness of Ashwagandha, Shatavari, and Trikatu, FEM-UP helps reduce androgen levels and PCOD-induced facial hair, aids in effective weight management, and boosts ovarian health, among numerous other benefits.

Remember, ignoring PCOD can cause a lot of serious complications, including:

- Type 2 diabetes
- Sleep apnea
- Endometrial cancer
- Infertility
- Miscarriage

Infused with the ages-proven goodness of all-natural ingredients, FEM-UP is the perfect supplement to promote overall women's health, correct irregular menses, and counter PCOD effects without having to worry about side effects. Sourced from nature's pharmacy of herbs, the ingredients in FEM-UP present the following benefits:

Ashwagandha

Improves sex drive. Provides relief from insomnia & fatigue

Shatavari

Helps restore PCOD-induced hormonal imbalance

Shankhpushpi

Helps improve digestion & reduce depression

Musli

Improves sexual performance & overall health

Brahmi

Reduces anxiety, depression & stress

Supplement Suggestions

Aparna Divakar

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

Deficiency/Out of Range Parameter(s)	Suggested Supplement
Cholesterol-Total, Serum	APPLE CIDER VINEGAR
SGOT/AST	LIV-UP

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



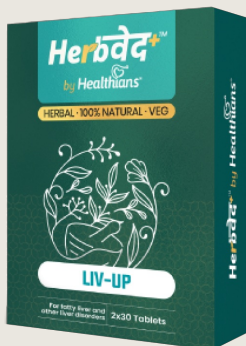
APPLE CIDER VINEGAR

Boosts Energy & Improves Immunity

Herbved Apple Cider Vinegar with Mother is 100% natural, raw, unfiltered and unpasteurized that has been fermented from Himalayan apples. It contains a substance called mother that consists of antioxidants, proteins, enzymes, and friendly bacteria that offers you a host of health benefits like losing weight, controlling High BP, Sugar & Cholesterol and improving digestion. It can work wonders on your hair making it soft and shiny, controlling dandruff & hairfall and can also prevent acne when applied as a toner externally on your skin.

Benefits of Apple Cider Vinegar:

Helps in weight loss | Helps control cholesterol levels | Lowers blood pressure | Controls hairfall and makes hair soft and shiny | Improves digestion



LIV-UP

De-toxify your body with a healthier liver.

LIV-UP is a scientifically formulated and clinically proven all-natural supplement that takes care of your liver and its functions. This ayurvedic supplement keeps your liver cool, and optimally functioning, thus promoting healthy digestion. Take the all-natural road to robust liver health with LIV-UP.

Untreated or unmanaged liver issues can cause grave and even lethal complications, which include:

• Liver Infections | • Liver Cancer | • Liver Failure | • Elevated Blood Toxin Levels | • Liver Cirrhosis

Infused with the ages-proven goodness of all-natural ingredients, LIV-UP is the perfect supplement to promote and maintain good liver health, without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in LIV-UP present the following benefits:

Methi Dana

Reducing the risk of developing fatty liver disease.

Triphala

Promotes liver function & boosts immunity

Ginger

Supports liver health & prevents liver inflammation

Yellow Myrobalan

Keeps the liver cool & promotes optimal functioning

Kulki

Reduces inflammation & protects the liver from injury

To order, call 1800-572-000-4

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**₹5499
ONLY**

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About Healthians Labs

How we control Report Accuracy at Healthians



Quality Control

We follow Quality control to ensure both **precision & accuracy** of patient results.



Machine Data

We save patient's result values **directly from machines** ensuring no manipulations & no fake values.



QR Code

QR Code based authenticity check on all its reports



Calibration

We make use of calibrators to evaluate the **precision & accuracy** of measurement equipment.



Equipment

Our Labs are equipped with state-of-the-art instruments with **cutting edge technology** to provide faster & reliable results.



EQA

Our Labs participate in EQA & show proven accuracy by checking **laboratory performance** through external agency or facility.

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